

ICD-0-3
carcinoma, infiltrating duct, NOS 8500/3
Site: breast, NOS C50.9
5/23/11

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	ML	Date Reviewed 5/19/11

ACC # 26

- A) ADDITIONAL MARGIN OF MOST MEDIAL TUMOR, EXCISION:
Benign breast tissue, no tumor present.
- (B) LEFT BREAST, SEGMENTAL MASTECTOMY:
MULTIFOCAL INVASIVE DUCTAL CARCINOMA WITH ASSOCIATED LYMPHOCYTIC INFILTRATE, HIGH NUCLEAR GRADE, NOTTINGHAM HISTOLOGIC GRADE 3. (SEE COMMENT)
LARGEST FOCUS OF INVASIVE CARCINOMA MEASURES 1.2 CM IN GREATEST DIMENSION IN THE MEDIAL ASPECT OF THE SPECIMEN.
MULTIPLE SATELLITE FOCI OF INVASIVE CARCINOMA ARE LATERAL TO THE MAIN FOCUS AND MEASURE 5 MM (X 2), 4 MM, 3 MM AND 1 MM.
LYMPHOVASCULAR INVASION IDENTIFIED.
DUCTAL CARCINOMA IN SITU (DCIS), HIGH GRADE, PREDOMINANTLY CRIBRIFORM AND CLINGING TYPES WITH CANCERIZATION OF LOBULES AND ASSOCIATED LYMPHOCYTIC INFILTRATE.
DCIS EXTENDS IN AN APPARENT IRREGULAR DISTRIBUTION OVER A 4 CM AREA.
DCIS EXTENDS TO LESS THAN 1 MM FROM THE ANTERIOR MARGIN, TO 1 MM FROM THE INFERIOR MARGIN, AND TO 2 MM FROM THE SUPERIOR AND POSTERIOR MARGINS.
INVASIVE CARCINOMA EXTENDS TO 2 MM FROM THE ANTERIOR MARGIN. (SEE COMMENT)
- (C) ADDITIONAL CENTRAL ANTERIOR INFERIOR MARGIN, EXCISION:
Benign breast tissue, no tumor present.
- (D) LEFT AXILLARY CONTENTS, DISSECTION:
METASTATIC CARCINOMA IN ONE OF THIRTY LYMPH NODES (1/30); METASTASIS MEASURES 7 MM; NO EXTRANODAL EXTENSION IDENTIFIED.
- (E) ADDITIONAL LEVEL I LEFT AXILLARY TISSUE, EXCISION:
Two lymph nodes, no tumor present (0/2).

Entire report and diagnosis completed by

COMMENT

Additional tissue was re-excised, and invasive carcinoma does not approach the final margins. However, I with involvement of lobules is 1 mm from the final inferior margin and 2 mm from the final superior and posterior margins.

Tumor marker studies will be performed and reported in an addendum.

UUID: CA6B733A-6030-422C-9DE9-B9718CBB2FDC
TCGA-GM-A2DF-01A-PR Redacted



GROSS DESCRIPTION

- (A) ADDITIONAL MARGIN OF MOST MEDIAL TUMOR - Received is a piece of fibroadipose tissue (1.8 x 1.2 x 0.5 cm) with clips on the true margin. The true margin inked black. The specimen is serially sectioned and entirely submitted in A.
- (B) LEFT SEGMENTAL MASTECTOMY - A segmental mastectomy specimen (8.0 x 6.5 x 5.5 cm) with a short stitch at the superior aspect, long stitch at the lateral aspect, and multiple clips on the anterior aspect. The specimen is serially sliced from medial to lateral into 11 slices. A well-circumscribed tan-pink tumor 1.4 x 1.1 x 1.0 cm, with an associated biopsy clip is identified in

slice #3, which is 0.7 cm from the closest anterior margin. Multiple smaller nodules are identified lateral to the main tumor. One of these is very close to the anterior margin in the central anterior inferior aspect of the specimen, and additional tissue is requested from this margin.

INK CODE: Anterior - yellow, posterior - black, superior - blue, inferior - green, medial and lateral - red.

SECTION CODE: B1, medial perpendicular margin; B2, tissue adjacent to tumor from slice #2; B3-B7, tumor and margins from slice #3 (B3 and B4, tumor with anterior margin in area of clip; B5, inferior margin; B6, posterior margin; B7, superior margin); B8, fibrous tissue adjacent to the tumor from slice #4; B9, one small nodule, 1.2 cm from superior margin and 1.8 from anterior margin of slice #4; B10, one small nodule 1.0 cm from anterior margin and 1.2 cm from superior margin of slice #5; B11, one small nodule, 1.5 cm from anterior margin, 2.0 cm from superior margin of slice #5; B12, B13, one small nodule, 0.1 cm from anterior margin and 1.2 cm from inferior margin of slices #6 and #7; B14, one small nodule 0.4 cm from posterior margin and 1.5 cm from inferior margin of slice #4; B15-B18, remainder of margins of slice #4, entirely submitted (B15, anterior margin; B16, superior margin; B17, posterior margin; B18, inferior margin); B19, representative section from slice #8; B20, lateral perpendicular margin.

(C) ADDITIONAL CENTRAL ANTERIOR INFERIOR MARGIN, LEFT BREAST - A portion of fibroadipose tissue (2.0 x 1.3 x 0.5 cm) with clips on the true margin. The true margin is inked black. The specimen is serially sectioned and entirely submitted in C1-C2.

(D) LEFT AXILLARY CONTENTS - Adipose tissue (11.0 x 7.5 x 3.0 cm). Multiple lymph nodes are identified ranging from 0.2 x 0.2 x 0.2 cm to 2.5 x 1.5 x 0.8 cm.

SECTION CODE: D1-D3, each containing six lymph nodes; D4, four lymph nodes; D5, four lymph nodes; D6, one lymph node, trisected; D7, one lymph node, serially sectioned; D8, one lymph node, serially sectioned; D9 and D10, one lymph node, serially sectioned.

(E) ADDITIONAL LEVEL I LEFT AXILLARY TISSUE - Two lymph nodes (0.2 x 0.2 x 0.2 cm and 0.8 x 0.4 x 0.4 cm), entirely submitted.

SECTION CODE: E1, one lymph node; E2, one lymph node, trisected.

CLINICAL HISTORY

Left breast cancer.

SNOMED CODES

T-04050, M-85003, M-85002 T-C4710, M-85006

"Some tests reported here may have been developed and performance characteristics determined by _____ These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Released by: _____

Start of ADDENDUM #1

ADDENDUM

Addendum completed by _____

This report is issued to give immunohistochemistry results.

Immunohistochemical staining is performed on a representative formalin-fixed, paraffin-embedded section of INVASIVE DUCTAL CARCINOMA, left breast, block B3.

MARKER	RESULTS	% POSITIVE	SCORE
Estrogen Receptor	Negative	0 %	N/A
Progesterone Receptor	Negative	< 1 %	N/A
HER-2/neu overexpression	Negative		Focal 1+

Due to the above HER-2/neu immunohistochemical staining result, gene copy level (HER-2/neu: CEP17 signal ratio) will be evaluated by FISH and a separate report will be issued.

FOOTNOTE

Estrogen receptor was assessed by immunohistochemistry using antibody 6F11 (

Progesterone receptor was assessed by immunohistochemistry using antibody PgR1294(

HER-2/neu was assessed by immunohistochemistry using antibody AB8 (I

Released by:

Start of ADDENDUM #2

ADDENDUM #2

This modified report is being issued to report the results of HER-2/neu FISH

Addendum completed by

SPECIMEN SOURCE

Left breast

SUMMARY

Tissue section of the invasive carcinoma B3 was evaluated for HER-2/neu gene amplification by interphase fluorescence in situ hybridization technique using the HER-2/neu DNA Probe Kit (LSI HER-2/neu /CEP17

Slide adequacy is satisfactory. Sixty tumor nuclei were counted and showed an average of 1.57 copies of LSI HER-2/neu gene per nucleus and an average of 1.40 copies of CEP17 per nucleus.

Negative and positive controls (established by appropriate.

) with this batch are

Two representative images have been archived.

INTERPRETATION

The tumor cells demonstrated **no amplification** of the HER-2/neu gene copy levels (HER-2/neu: CEP17 signal ratio: **1.12**)

The following guideline has been established for HER-2/neu testing:

Normal HER-2/neu levels: < 1.80; equivocal HER-2/neu levels: 1.80 – 2.20; amplified HER-2/neu levels: > 2.20

(Ref: Arch Pathol Lab Med. 2007; 131:18-43)

NOTES

The LSI HER-2/neu probe is specific for the HER-2/neu gene Locus (17q 11.2-q12) and the CEP 17 DNA probe is specific for the alpha satellite DNA sequence at the centromeric region of Chromosome 17 (17 p11.1 -q11.2).

This test has been cleared and approved for specific uses by the U.S. Food and Drug Administration. Its system is operating within the performance specifications stated in the product insert.

Released by: .

-----END OF REPORT-----